

If I wanted to lose belly fat FAST, here's what I'd do (copy me)

Channel: Brandon Carter · Published: Sep 15, 2026 · Watch: <https://www.youtube.com/watch?v=0vODOdYj4ts> Source: full transcript

Executive Summary

Brandon Carter, a 27-year personal trainer, outlines an aggressive fat-loss protocol: a deep caloric deficit (bodyweight × 10), 1g protein per pound of bodyweight, 20,000 daily steps, full-body weight training 3–4x/week, and optional peptides — all designed to burn fat while preserving muscle.

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Framing & Philosophy: Lose Fat, Keep Muscle 0:00

- Carter opens with a disclaimer: this is an **extreme protocol**, not a gentle beginner plan
 - He acknowledges it may feel unrealistic but challenges viewers not to argue for their own limitations
 - "If you argue for your own limitations, guess what? You get to keep them."
- The core goal is to lose fat while retaining muscle — not just to watch the scale drop
 - Starving yourself + exhausting workouts causes muscle loss, leaving you "skinny skinny" instead of lean and muscular
 - The target: a smaller waist with maintained muscle, resulting in a "heroic" physique
- Spot reduction is a myth
 - Crunches build abdominal muscle but do NOT burn belly fat
 - A better saying than "abs are made in the kitchen": *"abs come from being in the kitchen less"* — you must reduce overall body fat to uncover abs
- Beginners (first year of serious training) can build muscle AND lose fat simultaneously at this level of protein and training

- After the first year, achieving this naturally becomes much harder
- Three pillars of the protocol:
 - Diet (aggressive deficit + high protein)
 - Walking (a lot of it)
 - Lifting (specific style to maintain muscle)

Step 1 — Set an Aggressive Caloric Target 0:265

- **Formula: Body weight in pounds × 10 = daily calorie target**
 - Example: 200 lb person eats 2,000 calories per day
- Why NOT a 300–500 calorie deficit (as many influencers recommend):
 - Real life is messy — eyeballing olive oil, underestimating salad dressing, bites of kids' food all add up
 - Carter personally weighs his condiments everywhere, even at upscale restaurants like Mastros — but acknowledges most people won't do this
 - One cheat meal on Saturday can wipe out a small planned deficit entirely
 - "The smaller your planned deficit is, the easier it is for normal human effort to completely mess it up"
- The bodyweight × 10 formula creates a deficit large enough to produce dramatic results AND absorb real-world tracking errors
- Simplify the diet drastically:
 - Pick a handful of meals whose calories you already know, that keep you full, and that you enjoy — then eat those repeatedly
 - People in the best shape tend to eat the same foods most of the time
 - Carter posts his (nearly identical) daily meals on Instagram (@KingKeto)
- Foods to eliminate — they add high "accidental" calories:
 - Nut butters
 - Nuts
 - Cooking oils
 - Fancy coffees
 - Random snacks
 - Alcohol
 - "Healthy" smoothies (some contain ~900 calories)

Step 2 — Prioritize Extremely High Protein 0:621

- **Formula: 1 gram of protein per pound of CURRENT body weight (not goal weight)**
 - 200 lb person = 200g protein per day
 - Carter himself consumes 150–300g protein daily
- Why such high protein? Three distinct reasons:

- **Muscle preservation:** In a deep deficit, the body risks catabolizing muscle; high protein signals the body to retain it
 - The common "science-based" recommendation of 0.7–0.8g/lb is for muscle *building*, not muscle *preservation in a deficit* — you need more when cutting hard
- **Thermogenic effect of food (TEF):** The body burns calories during digestion, and protein has the highest TEF of any macronutrient
 - **20–30% of protein calories are burned during digestion** — twice as much as carbs, three times as much as fat
 - Eating 100 calories of protein means you only absorb ~70–80 of those calories
 - Carter's analogy: protein is like a cashback credit card — you get some back on every purchase
 - This effectively deepens your deficit beyond the stated calorie number
- **Satiety:** High protein keeps you fuller longer, reducing the urge to overeat
- Protein within the overall calorie budget (bodyweight × 10) makes the deficit even more effective because of TEF

Step 3 — Walk 20,000 Steps Per Day 0:935

- **Target: 20,000 steps per day**
- Why walking specifically (not sprinting or intense cardio):
 - You're already lifting weights AND in a deep caloric deficit
 - Walking burns significant calories without turning every session into a near-death experience
 - Low-impact — sustainable alongside the rest of the protocol
- Exact daily schedule to hit 20,000 steps:
 - 30 minutes immediately after waking up → ~3,000 steps
 - 30 minutes after lunch → ~3,000 steps
 - 60 minutes after work → ~6,000 steps
 - **Running total: ~12,000 steps**
 - Set an alarm every hour at work and walk for 10 minutes → ~1,000 steps per break
 - 8 breaks throughout the workday → ~8,000 additional steps
 - **Grand total: ~20,000 steps — spread across the day, not one giant block**
- This is temporary: "Do it till you're not fat anymore" — Carter himself now averages 10,000–15,000 steps/day at maintenance
- Tracking options:
 - Wearables are more accurate than phone alone
 - Carter uses an Oura Ring; also mentions Apple Watch, Whoop, Samsung watch
- Tools Carter uses: walking desk (treadmill desk) for work calls and meetings; morning walks

Step 4 — Full-Body Weight Training 1:196

- **Purpose of lifting during a cut: give the body a reason to HOLD ON to muscle** — not to burn calories (that's the diet and walking's job)
- Frequency: **3–4 full-body workouts per week**
 - Carter personally does 5–6 (nearly 30 years of training = more volume tolerance)
 - 3–4x is the right starting point for most people
 - Carter has done full-body workouts exclusively for 11–12 years and considers them far superior to splits
- Structure of each workout:
 - 1–2 exercises per body part (chest, back, shoulders, triceps, legs)
 - 3–4 sets per exercise
 - Rest: **60–90 seconds between sets — timed strictly, no exceptions**
 - Workout duration: **45–60 minutes maximum** (not 2–3 hours)
 - Reason workouts run long: wasting time on TikTok, talking, not timing rest periods
 - Carter uses Apple Watch Ultra voice command ("Set 60-second timer") immediately after every set
- Superset strategy:
 - Train opposing muscle groups back-to-back (e.g., chest set → back set → 90-second rest; curls → tricep extensions → rest)
 - Increases efficiency without compromising performance
- Carter personally splits his workouts into two 30-minute or three 20-minute sessions (possible because he has a gym in his condo)
- **Critical mistake to avoid:** Turning weightlifting sessions into cardio during a cut
 - High-rep "cardio" lifting (30 reps, jump squats between sets) gives the body LESS reason to maintain muscle
 - Lift heavy and hard; let the deficit and walking handle fat loss
- Ab training:
 - Train abs 1–2 times per week like any other muscle group
 - Ab training builds the muscle but does NOT burn the overlying fat
 - Carter's top 3 ab exercises (highest bang for buck):
 - Cable crunches
 - Hanging leg raises
 - Ab wheel rollouts

Step 5 — Supplements & Peptides 1:548

- **Disclaimer repeated multiple times:** Talk to your doctor before taking anything, especially the peptides below.

Supplement	Dose Mentioned	Key Benefit
Creatine Monohydrate	5g (standard); Carter takes 20g	Muscle retention; at 20g may improve cognition
Caffeine	FDA says ~400mg/day is safe; Carter takes 1,000mg (not recommended to copy)	Appetite suppression, energy, weight loss support
Tirzepatide (GLP-1)	Doctor prescribed	Dramatically suppresses hunger; made cutting "the easiest cut I've ever done"
Tesamorelin	Doctor prescribed	Growth hormone secretagogue; FDA-approved to reduce abdominal fat (tested in HIV patients); Carter reports visible veins in abs and significant definition
MK-677 / MOD-GRF ("Mod C")	Not specified	Increases mitochondrial function; noticeable energy boost — called "exercise in a bottle"

- **Creatine:** Carter has taken it since age 16 (now ~44); new data suggests 5g is sufficient for muscle building but 20g may enhance cognition
- **Caffeine:** Carter takes 1,000mg daily for 20+ years with perfect EKG results — but stresses individual variation is enormous and this dose would likely cause panic attacks in most people. FDA-safe upper limit is ~400mg.
- **Tirzepatide (GLP-1 agonist):**
 - Carter's experience: hunger was "zapped" completely, the cut felt almost too easy
 - Risk of undereating when on a GLP — must still track calories and hit protein targets or risk muscle loss and a "skinny fat" look
 - Carter's take: "Dieting without a GLP is making things more difficult for no reason" — compares skipping it to refusing to use a car or the internet when better tools exist
 - Note on Retatrutide: Carter hasn't tried it because it's not legally available from a doctor in the US; also, some users report elevated heart rate as a side effect
- **Tesamorelin:**
 - FDA-approved peptide; stimulates the body to secrete more growth hormone
 - Originally tested and approved to reduce visceral abdominal fat in HIV patients
 - Carter's analogy: like Adderall for a regular person — if it helps someone struggling, imagine what it does for someone already healthy
 - Also reduces visceral fat (fat around organs) — "the stuff that's killing people"
 - Carter reports unprecedented ab definition and vascularity since starting it
- **"Mod C" peptide:**
 - Increases mitochondrial function → more energy, especially valuable in a caloric deficit

- **Scale weight fluctuates daily — do not panic**
 - Water retention, sodium intake, bowel movements all cause day-to-day swings unrelated to fat gain/loss
 - Carter hid the scale from his girlfriend to protect her mental health
 - Weigh yourself every morning under identical conditions: after using the bathroom, before eating or drinking
 - **Take the 7-day average** — that's the number that matters
- **Measure your waist once per week** (at the belly button for consistency)
 - "The tape measure don't lie" — the scale can fluctuate; waist measurements reveal the real trend
- Take **weekly progress photos**: same lighting, same angle, same time of day
- **Commit to 30 days before changing the plan**
 - If there's no movement after **14 days**, do a full audit before blaming the protocol:
 - Are you accurately tracking calories?
 - Are you hitting your steps every day including weekends?
 - Are you slipping up on weekends (alcohol, cheat meals, estimating portions)?
 - "Check the math before you start blaming your metabolism."
- **What "fast" actually means:**
 - The first few days will show dramatic weight drops — mostly water and glycogen, not pure fat (especially if starting very overweight)
 - The fatter you are, the faster initial loss; the leaner you get, the slower fat loss becomes — this is normal, not failure
 - Men store more fat cells in the lower abdomen and love handles — those areas are last to lean out
 - Having the top 2 abs but not the bottom 4 doesn't mean something is wrong — "you just not done yet"
 - Don't plan-hop (carnivore, juice cleanse, etc.) — stay consistent and trust the process

Full Protocol Summary 2:309

- **Calories:** Body weight (lbs) × 10 = daily calorie target
- **Protein:** 1g per pound of current body weight
- **Diet simplicity:** Pick a few meals you know the calories for; eat them repeatedly; eliminate high-calorie-density foods (nuts, oils, alcohol, fancy coffees, smoothies)
- **Steps:** 20,000 per day
 - 30 min walk on waking (~3,000 steps)
 - 30 min walk after lunch (~3,000 steps)
 - 60 min walk after work (~6,000 steps)
 - 10-min walking breaks every hour at work × 8 (~8,000 steps)
- **Lifting:** Full-body workouts 3–4x per week
 - 1–2 exercises per body part

- 3–4 sets per exercise
- 60–90 second rest periods (timed)
- Workouts $\leq$ 60 minutes
- Lift hard — do NOT turn sessions into cardio
- **Abs:** Train 2–3x per week (cable crunches, hanging leg raises, ab wheel)
- **Supplements:** Creatine (~5g), caffeine (~400mg FDA limit), and consult a doctor about tirzepatide, tesamorelin, and Mod C peptides
- **Tracking:** Daily weigh-ins (7-day average), weekly waist measurement, weekly progress photos
- **Commitment:** Stick to the plan for 30 days; audit after 14 days if no progress; don't switch programs

The entire system in one line: **Caloric deficit (bodyweight $\times$ 10 cals) + high protein (1g/lb) + 20,000 steps + heavy full-body lifting = lose the fat, keep the muscle.**